

PROGRAM SERVICES AGREEMENT

THIS PROGRAM SERVICES AGREEMENT (the “**Agreement**”) is entered into and effective as of March 12, 2026 (“**Effective Date**”), by and between by and between CATCH Greater Houston, a Texas not-for-profit corporation (“**CATCH**”) and **Ibn Sina Foundation** (“**Clinic**”). CATCH and Clinic are sometimes referred to herein as, individually, a “**Party**” and, collectively, the “**Parties**.” Capitalized terms not otherwise defined in this Agreement shall have the meanings ascribed to them in the **Program Appendix** (attached hereto).

RECITALS

WHEREAS, Clinic is a medical practice servicing communities in and around Houston, Texas, offering comprehensive primary and specialty care services;

WHEREAS, CATCH is a community care hub that has developed evidence-based provider enablement and care navigation support programs (the “**Program**”), that include proprietary delivery protocols, personalized techniques, patient navigation, care management, and provider training to enhance access to care and improved health outcomes, delivered in conjunction with primary care settings that furnish comprehensive primary care services;

WHEREAS, in order to the care delivery requirements of the Program, CATCH desires to engage Clinic to participate in the Program as a Local Health Clinic Partner, pursuant to terms and conditions expressed herein, to perform the obligations and responsibilities of a Local Health Clinic Partner as set forth in the **Program Appendix** (“**Services**”);

AGREEMENT

NOW, THEREFORE, in consideration of the mutual covenants and agreements contained herein, the Parties agree as follows:

1. **Definitions.** The following capitalized terms used herein shall have the meanings set forth below:

- a. “**Clinician**” means a licensed healthcare professional employed by or, directly or indirectly, contracted with Clinic, who will furnish Services hereunder.
- b. “**Payor Agreement**” means any agreement or contract under which Clinic receives payment for professional services provided to Payor’s beneficiaries, enrollees or other covered persons.
- c. “**Payor**” means any government health care program (including Federal Health Care Program) or any insurance company, managed care organization, health or medical plan or program or other third-party payor, whether private, commercial, employer or governmental, or any fiscal intermediary or contractor of any of the foregoing.

2. **Program Participation.** CATCH hereby engages Clinic, during the term of this Agreement, as a Local Health Clinic Partner to provide the Services, and Clinic hereby accepts such engagement and agrees to participate in the Program as a Local Health Clinic Partner and to

furnish the Services in accordance and compliance with the terms of this Agreement. The Program is described in the **Program Appendix** and subject to the following requirements. Any capitalized terms used, but not defined, in this Section 2 shall have the meanings ascribed to them in the **Program Appendix**.

- a. **Records.** Clinic shall prepare and maintain complete, timely, and accurate (i) records for Services that are professional medical services in Clinic's HIPAA-compliant electronic medical record system ("EHR") after Services are performed and (ii) fiscal and administrative records, in each case, in accordance with Clinic's policies and procedures, if any, and customary health care industry and recordkeeping standards and with applicable law, and shall retain such records for a period of six (6) years or the period prescribed by applicable law, whichever is longer.
- b. **Claims Billing.** Clinic has the exclusive right and responsibility for the billing and collection from Payors for Clinical Services provided or supervised by Clinic and/or its Clinicians to Program Members, including, without limitation, under Clinic's Payor Agreements.
- c. **Nondiscrimination.** Neither Party shall, and Clinic shall ensure its Clinicians shall not, discriminate against any Clinic Member in the provision of Services, whether on the basis of the Program Member's coverage under a benefit plan, age, sex, marital status, sexual orientation, race, color, religion, ancestry, national origin, disability, handicap, health status, source of payment, utilization of medical or behavioral health services or supplies, retaliation, or other protected classification recognized by Title 45 of the Code of Federal Regulations, § 80.3-80.4, Civil Rights Act of 2007, or any unlawful basis.
- d. **Compliance with Third-Party Payor Requirements.** Clinic shall comply, and shall cause each Clinician to comply, with all applicable requirements of Payor Agreements and Federal Health Care Programs relating to the provision of the Program Services under this Agreement. Clinic shall use commercially reasonable efforts to notify CATCH in writing (a) at least one hundred twenty (120) days' prior to the effective date of termination, nonrenewal, or cancellation of any Payor Agreement pertaining to the Program; and (b) at least sixty (60) days prior to the effective date of any material amendments to a Payor Agreement pertaining to the Program wherein the scope of Services covered under the Payor Agreement is modified.
- e. **Quality Assurance.** Clinic shall participate and shall cause Clinicians to participate in quality improvement activities reasonably requested or conducted by CATCH in connection with the performance of the Program.
- f. **Program Member Complaints and Grievances.** With respect to Program Members receiving Clinical Services, Clinic will cooperate fully with CATCH in the investigation of Program Member complaints and grievances by providing information necessary for review and resolution of such complaints and grievances. The information will be made available in a timely manner according to Clinic established policies and procedures. CATCH will collaborate in the resolution of identified

opportunities for improvement without retaliation against the Program Member and in keeping with the wellbeing of such Program Member.

- g. **Patient Consents.** Clinic will ensure that all necessary consents have been secured from each Program Member, or his or her personal representative as appropriate, to (i) receive services from Clinicians and (ii) authorize Clinic to share patient health data, including Protected Health Information, with CATCH and Clinic's other staff and personnel. Each such consent will be made available to CATCH on request.

3. **Compensation.** As full and complete consideration for all services provided by the Parties under the Agreement and the applicable Program Appendix, the compensation payable under this Agreement shall be determined solely as set forth in the applicable Program Appendix (the "**Compensation**").

- a. **Taxes.** Clinic shall be responsible for all sales, use and excise taxes, and any other similar taxes, duties and charges of any kind imposed by any federal, state or local governmental entity on the Services provided by Clinic to or for CATCH and on any amounts payable by CATCH hereunder; provided, that, in no event shall Clinic pay or be responsible for any taxes imposed on, or regarding, CATCH's income, revenues, gross receipts, personnel, or real or personal property or other assets.

- b. **Compliance; Referrals.** Each Party agrees to fully comply with all applicable federal, state, and local codes, ordinances, rules, regulations, and requirements now in force, or which may hereafter be in force, affecting such Party's performance under this Agreement. The Parties intend this Agreement to comply with 42 U.S.C. § 1320a-7b(b) (the Federal Anti-Kickback Statute), 42 U.S.C. § 1395nn (the Stark Law) and any other federal or state law provision governing health care fraud and abuse.

- c. **No Inducements to Deny or Limit Care.** The compensation payable by Clinic for the Services provided herein is not a payment for the denial of medically necessary services to any Program Member. Each Party acknowledges and agrees that the compensation due to CATCH hereunder is not made as an offer or acceptance, directly or indirectly, of any form of inducement to reduce or limit the provision of medically necessary goods or services for any Program Member.

- d. **Consideration.** The Parties acknowledge and agree that, except as expressly set forth in this Agreement and the applicable Program Appendix, neither Party shall owe any additional fees or compensation to the other in connection with the Program. As All compensation arrangements under this Agreement are intended to be commercially reasonable and consistent with fair market value, and not determined in a manner that takes into account the volume or value of referrals or other business generated between the Parties, except as permitted by applicable law.

4. **Term and Termination.**

- a. **Term.** The initial term of this Agreement shall be for a period of two (2) years from the Effective Date ("**Initial Term**"). Upon the conclusion of the Initial Term, this Agreement shall automatically renew for subsequent successive one (1) year terms unless and until terminated in

accordance with Section 4.b (Termination) below. Notwithstanding anything to the contrary in the foregoing, this Agreement may be terminated as follows:

b. Termination.

i. **Termination Without Cause.** Any Party may terminate this Agreement at any time without cause upon one hundred and eighty (180) days' written notice.

ii. **Termination For Cause; Remedial Action.** Either Party may terminate this Agreement for cause upon written notice to the other Party in the event of a material breach by such Party, provided such material breach continues uncured for thirty (30) days after receipt by breaching Party of written notice from non-breaching Party specifying such breach.

iii. **Immediate Termination.** Any Party may terminate this Agreement immediately by written notice to the other Party upon the occurrence of any of the following events: (i) a Party is listed by a federal agency as being debarred, excluded, terminated, or otherwise ineligible for federal healthcare program participation; (ii) a Party discontinues operations for at least fifteen (15) days or loses its required licensure or accreditation; or (iii) a Party files a petition in bankruptcy, or makes an assignment for the benefit of creditors, or if any involuntary petition in bankruptcy or petition for an arrangement pursuant to any bankruptcy laws is filed against such Party and such petition or assignment is not dismissed or rescinded, as applicable, within ninety (90) days.

iv. **Obligations on Termination.** The Parties shall cooperate to ensure the smooth transition of patient care of Program Members to alternative provider, supplier or organization identified by CATCH upon termination of this Agreement. Within thirty (30) days of the termination or expiration of this Agreement, (A) each Party shall return to the other all equipment, software, and confidential information owned by the other Party or its agents; and (B) Clinic shall furnish to CATCH all data required by CATCH pursuant to this Agreement.

5. Duties and Responsibilities of Clinic.

a. Clinic shall participate in and support the Program as a Local Health Clinic Partner and provide Services as set forth in **Program Appendix**, and any other services that may become necessary to the extent agreed upon in a written amendment to this Agreement. CATCH shall not have nor exercise any control or direction over Clinicians' professional judgment in connection with the provision of any medical services provided hereunder.

b. Clinic shall use best efforts to provide general cooperation to CATCH regarding the operation of the Program and Services as provided herein.

c. Client shall comply with all applicable laws and regulations in performing this Agreement.

d. Clinic shall ensure that Services provided to Program Members are rendered in a competent, professional, and ethical manner, and in accordance with applicable standards of care

and scope of practice. Clinic warrants, represents, and agrees to ensure that Services requiring licensure, registration, or certification are delivered by providers with such qualifications, and that all Services delivered to Program Members, or otherwise under the Program, are delivered by appropriately educated, trained, experienced, and competent personnel.

e. Clinic shall notify CATCH within seven (7) days of becoming aware that Clinic, or any individual providing Services, is under investigation or has been sanctioned by the government or any licensing authority (including, without limitation, the imposition of program exclusion, debarment, civil monetary penalties, corrective action plans, revocation of Medicare billing privileges, and inclusion on the CMS preclusion list as defined at 42 C.F.R. § 422.2).

f. To ensure the submission of accurate, complete and truthful data and other information, Clinic agrees to make its internal practices, books, and records relating to this Agreement available to CATCH, upon reasonable request and reasonable notice, subject to Clinic's reasonable conditions, for purposes of auditing compliance with this Agreement.

6. **Duties and Obligations of CATCH.**

a. CATCH shall operate the Program as set forth on Program Appendix.

b. CATCH shall ensure Clinic has access to all applicable Program-specific protocols, policies, and procedures necessary to provide the Services, and shall furnish Clinic, as appropriate, with all information reasonably required by Clinic to perform the Services under this Agreement.

c. Notwithstanding anything to contrary herein, CATCH does not guaranty or warrant any referrals of Program Members or priority referral treatment to or among Local Health Clinic Partners, in each case, pursuant to the Program.

7. **Mutual Representations.** The Parties represents as follows, and shall notify the Party promptly upon becoming aware any of the representations below is no longer correct:

a. Each Party has the power and authority to enter into this Agreement and to perform its obligations hereunder.

b. No Party is not bound by any agreement or arrangement that would preclude such Party from entering into, or from fully performing the Services or its obligations, as applicable, required under this Agreement; and

c. No Party nor any individual providing Services: (i) is a "sanctioned person" under any federal or state program or law; (ii) has been listed in the current List of Excluded Individuals and Entities by the Office of Inspector General for the U.S. Department of Health and Human Services; (iii) has been listed on the General Services Administration's List of Parties Excluded from Federal Programs; or (iv) has been convicted of a criminal offense related to health care.

8. **Privacy & Security; Patient Records.** Each Party shall comply with all applicable federal and state laws and regulations relating to the maintenance, uses, and disclosures of protected health information, including without limitation the Health Insurance Portability and Accountability Act of 1996, as codified at 42 U.S.C. § 1320d through d-8 ("**HIPAA**"), as amended by the Health

Information Technology for Economic and Clinical Health Act, as incorporated in the American Recovery and Reinvestment Act of 2009, and any current and future regulations promulgated thereunder, including those published at 45 C.F.R. Parts 160, 162, and 164, all collectively referred to herein as “**HIPAA Requirements**.” In providing the Services herein, CATCH is a “**Business Associate**” of Clinic as such term is defined under the provisions of the HIPAA Requirements, and the provision of Services hereunder shall be subject to the terms of a Business Associate Agreement attached hereto as **Exhibit B**, and incorporated by reference herein. Except to the extent prohibited by applicable law, all patient records prepared by Clinic and its personnel in the performance of the Services hereunder are the non-exclusive property of Clinic, and shall, upon termination or expiration of this Agreement for any reason, remain the non-exclusive property of Clinic.

9. **Intellectual Property.** Ownership of inventions, technologies, know-how, ideas, processes, techniques, algorithms, platforms, programs, discoveries, improvements, devices, biologics, products, concepts, designs, prototypes, samples, models, technical information, materials, drawings, specifications, and other works of authorship existing as of the Effective Date hereof, and all patents, copyrights, trade secret rights, and other intellectual property rights therein (collectively, “**Pre-existing Intellectual Property**”), is not affected by this Agreement, and no Party shall have any claims to or rights in any Pre-existing Intellectual Property of the other Party or its agents, except as may be otherwise expressly provided in any other written agreement between the Parties. Any improvements to or derivatives of Pre-existing Intellectual Property, created after the Effective Date, shall be owned exclusively by the Party that owns the relevant Pre-existing Intellectual Property. Ownership of all other inventions shall be determined in accordance with U.S. inventorship law. This section will remain in full force and effect following any termination or expiration of this Agreement.

10. **Publicity; Use of Name.** Clinic grants to CATCH a limited, non-exclusive, royalty-free, and non-transferable right and license to use Clinic’s trade name and logo (collectively, the “**Licensed Intellectual Property**”) for the purpose of identifying the entity as a local health clinic partner in media, marketing, and other public communications specific to the Program; provided that Clinic shall approve all such written materials (which approval shall not be unreasonably, withheld or delayed). Clinic retains all right, title, and interest in and to its Licensed Intellectual Property. CATCH shall not acquire any right of any kind in the Licensed Intellectual Property as a result of its use of it.

11. **[Reserved.]**

12. **Confidentiality.** During the term of this Agreement, Clinic may become privy to or aware of proprietary or confidential information of CATCH or its agents, and vice versa, including, but not limited to, information relating to the professional and business practices of the disclosing Party, patients and the pricing of services, supplies and equipment. The receiving Party agrees to hold the disclosing Party’s confidential and proprietary information, including the terms of this Agreement, strictly confidential and shall not disclose it to any other party without first obtaining the written consent of the disclosing Party, except in the ordinary course of business to its directors, officers, employees, and agents of the receiving Party who have a need to know and have agreed to maintain the confidentiality of such matters. The disclosing Party’s confidential information shall remain the sole and exclusive property of that Party. Due to the unique nature of the services

it has developed and offers, CATCH's confidential information also includes, but is not limited to, CATCH's Pre-existing Intellectual Property, financial information, plans, business information, financial information, algorithms, protocols, materials and design of various elements of any relevant Services-related systems which may be seen or explained, CATCH's virtual care systems as a whole, the methods of operation thereof and the various applications thereof, trade secrets, and all other information that might reasonably be considered confidential, secret, sensitive, proprietary, or private whether communicated in writing or verbally. Notwithstanding the foregoing, nothing in this Agreement shall prohibit CATCH from using patient information or clinical or performance data obtained in connection with this Agreement for its own business purposes provided all such information is used in accordance with HIPAA Requirements.

a. **Exceptions; Compelled Disclosures.** No Party shall have any obligation with respect to confidential information which: (i) is known or used by the receiving Party prior to disclosure by the disclosing Party; (ii) either before or after the date of the disclosure by the disclosing Party, is disclosed to the receiving Party by a third Party under no obligation of confidentiality to the disclosing Party; (iii) either before or after the date of the disclosure to the receiving Party, becomes published or generally known to the public through no fault of the receiving Party; (iv) is independently developed by the receiving Party; (v) is required to be disclosed by a final order of a court of competent jurisdiction; or (vi) is otherwise required to be disclosed by applicable law following reasonable notice to the disclosing Party. If any Party is requested or compelled to disclose any confidential information, such Party shall provide the disclosing Party with prompt written notice of such request(s) to allow the disclosing Party to seek a protective order or remedy or consent to the disclosure. If a protective order is not obtained, or compliance with the provisions of this Agreement is waived, the Party making disclosure agrees that it shall disclose only such information that is legally required and shall exercise its best efforts to obtain reliable assurances that confidential treatment shall be accorded to such information that is being disclosed.

b. **Injunctive Relief.** Any use or disclosure of the disclosing Party's confidential information other than as specifically provided for in this Agreement may result in irreparable injury and damage to the disclosing Party. In the event of use or disclosure by the receiving Party other than as specifically provided for in this Agreement, the disclosing Party may be entitled to equitable relief as granted by any appropriate judicial body. Upon written request of the disclosing Party hereto, the receiving Party shall promptly return to such disclosing Party or destroy any confidential information in its possession and certify in writing to the disclosing Party that it has returned or destroyed all such confidential information.

13. **Insurance.** During the Term of this Agreement, the Parties shall each, at its own expense, maintain and carry commercially reasonable insurance with financially sound and reputable insurers, in full force and effect that includes, but is not limited to (i) commercial general liability insurance in amounts not less than \$1,000,000 per occurrence and \$2,000,000 in the aggregate, or in any greater minimum amount if required by applicable law; (ii) cyber liability insurance covering claims based on a violation of any applicable law concerning the privacy of patient information and claims based on obligations pursuant to this Agreement with coverage of not less than \$1,000,000 per occurrence; (iii) professional liability insurance and errors and omission insurance with coverage limits that are reasonable based on industry standards, but not less than \$2,000,000 per occurrence and \$2,000,000 in the aggregate, applicable to incidents arising out of

and related to such of the Parties' respective obligations under this Agreement; (iv) as well as all insurance required by applicable law. Upon request, a Party shall provide the requesting Party with a certificate of insurance from its insurer evidencing the insurance coverage specified in this Agreement.

14. **Indemnification.** Each Party shall indemnify and hold harmless the other Party and its officers, directors, employees, agents, representatives, successors, and permitted assigns ("Indemnified Persons"), from any and all claims asserted against the indemnified Party by any person who is not a Party to this Agreement (and all related liabilities, costs, and expenses of any kind whatsoever, including but not limited to attorneys' fees and court costs), resulting from (i) the negligent acts or omissions or misconduct of the indemnifying Party, its employees or agents; (ii) noncompliance with any applicable federal or state law, rule or regulation by the indemnifying Party, its employees or agents; or (iii) any breach by the indemnifying Party, its employees or servants of any material term of this Agreement; provided, however, that the indemnifying Party's obligations under this Section 14 shall not apply to claims which arise from or are alleged to arise from: (A) a failure by the indemnified Party to materially adhere to the terms of this Agreement, (B) a material breach of any applicable federal or state law, rule or regulation by the Indemnified Persons, or (C) the indemnified Party's negligence or willful or wanton misconduct. The obligations herein shall survive termination or expiration of this Agreement. The foregoing indemnification obligation is contingent upon the indemnified Party promptly notifying the indemnifying Party in writing of any such claim (but any delay in notification shall not release the indemnifying Party from such indemnification obligation unless it is prejudiced) and on the indemnified Party providing the indemnifying Party with reasonable assistance (at the indemnifying Party's sole expense) in connection therewith.

15. **LIMITATION OF LIABILITY.**

a. IN NO EVENT SHALL EITHER PARTY BE LIABLE TO THE OTHER PARTY OR TO ANY THIRD PARTY FOR ANY LOSS OF USE, REVENUE, OR PROFIT OR LOSS OF DATA OR DIMINUTION IN VALUE, OR FOR ANY CONSEQUENTIAL, INCIDENTAL, INDIRECT, EXEMPLARY, SPECIAL, OR PUNITIVE DAMAGES WHETHER ARISING OUT OF BREACH OF CONTRACT, TORT (INCLUDING NEGLIGENCE), OR OTHERWISE, REGARDLESS OF WHETHER SUCH DAMAGE WAS FORESEEABLE AND WHETHER OR NOT EITHER PARTY HAS BEEN ADVISED OF THE POSSIBILITY OF SUCH DAMAGES, AND NOTWITHSTANDING THE FAILURE OF ANY AGREED OR OTHER REMEDY OF ITS ESSENTIAL.

b. IN NO EVENT SHALL EITHER PARTY'S AGGREGATE LIABILITY ARISING OUT OF OR RELATED TO THIS AGREEMENT OR ANY PROGRAM, WHETHER ARISING OUT OF OR RELATED TO BREACH OF CONTRACT, TORT (INCLUDING NEGLIGENCE), OR OTHERWISE, EXCEED THE AMOUNT PAID TO CLINIC HEREUNDER FOR THE SIX (6) MONTH PERIOD PRIOR TO THE EVENT THAT GAVE RISE TO THE ALLEGED LIABILITY.

16. **Compliance Amendment.** If a Party receives notification (the "**Notification**") of: (i) an opinion (the "**Opinion**") of nationally recognized health care counsel selected by that Party that it is more likely than not that applicable legislation, regulations, rules, or procedures (collectively

referred to herein as a “**Law**”) in effect or to become effective as of a date certain, or (ii) an actual or threatened decision, finding, or action by any governmental agency or court (collectively referred to herein as an “**Action**”), which Law or Action, if or when implemented, would have the effect of subjecting any Party to civil or criminal prosecution, or other material adverse proceeding or impact, under state and/or federal laws governing health care fraud and abuse, then the Party receiving the Notification shall provide the Notification to the other Party. The Parties shall attempt in good faith to amend this Agreement to the minimum extent necessary in order to comply with such Law or to avoid the Action, as applicable, and shall utilize mutually agreed upon joint legal counsel to the extent practicable. If, within thirty (30) days of providing written notice of the Notification to the other Party, the Parties hereto acting in good faith are unable to mutually agree upon and make amendments or alterations to this Agreement to meet the requirements in question, or alternatively, the Parties mutually determine in good faith that compliance with such requirements is impossible, unfeasible or unacceptable, then this Agreement shall be terminated at any Party’s election, without penalty, charge, or continuing liability, upon the earlier of the following: (i) the date thirty (30) days subsequent to the date upon which the terminating Party gives written notice to the other Party, or (ii) the effective date upon which the Law or Action prohibits the relationship of the Parties pursuant to this Agreement.

17. **Miscellaneous.**

a. **Independent Contractors.** CATCH and Clinic are independent contractors with respect to each other. Nothing under this Agreement is intended, nor shall be construed, to create a partnership, employer-employee relationship, or joint venture between the Parties. No Party (nor its respective agents or employees) shall have any authority to bind any other Party without such other Party’s express written consent. Each Party shall be solely responsible for reporting, withholding, and paying all taxes and other sums due to individuals providing services on its behalf. Likewise, each Party shall be solely responsible to provide and pay for all workers’ compensation insurance for any employed individuals providing services on its behalf.

b. **Entire Agreement; Amendment.** This Agreement sets forth the entire agreement between the Parties with regard to the subject matter hereof, and supersedes all prior or contemporaneous oral or written agreements entered into between the Parties. This Agreement and Exhibits may be amended only by a written agreement signed by both Parties.

a. **Governing Law.** This Agreement, as well as all disputes arising hereunder or related thereto, shall be governed by, and construed and enforced in accordance with, the laws of the State of Texas without regard to conflict of law principles. The exclusive forum for the foregoing is in the state or federal courts in Harris County, Texas, unless such action cannot by law be brought in such forum, in which case the venue required by law shall govern

c. **Assignment; Subcontracting; Binding Effect.** Clinic may not assign or transfer this Agreement, including by merger, consolidation, dissolution, or otherwise, nor may Clinic subcontract or delegate its responsibilities under this Agreement to any entity or organization, without the prior written consent of CATCH, and any attempt to do so shall be void ab initio. Subject to the foregoing, this Agreement shall inure to the benefit of and bind the Parties hereto and their respective heirs, representatives, successors, and permitted assigns.

d. **Severability.** The invalidity or unenforceability of any term or provision of this Agreement shall not affect the validity or unenforceability of any other term(s) or provision(s).

e. **Waiver.** No waiver, including any waiver of breach, shall be valid unless in writing and signed by the Parties.

f. **Recitals/Captions.** The recitals are incorporated into this Agreement, but the captions are used solely for convenience.

g. **Force Majeure.** With the exception of delays in payment, each Party shall be excused from delays in performing its obligations if resulting from causes beyond the commercially reasonable control of such Party, including default of vendors, acts of God, governmental actions, fire, flood, epidemic, and embargoes.

h. **Notices.** All notices hereunder shall be in writing, delivered personally, by certified or registered mail, return receipt requested, by overnight courier, or by facsimile or e-mail of a PDF document (if sent during normal business hours of the recipient) and shall be deemed to have been received when delivered in person or as of the date recorded on a signature card or similar proof of receipt, to the address below, or to such other persons or addresses as a Party may provide by written notice to the other Parties. Any Party may change its notice address by notice to the other Parties given in the manner set forth above.

If to Clinic:
Ibn Sina Foundation
ATTN: CEO
11226 S. Wilcrest Dr.
Houston, TX 77099

If to CATCH:

CATCH Greater Houston
ATTN: Janice Sparks
190 Heights Blvd.
Houston, TX 77007

With a copy to (which shall constitute notice):
Foley & Lardner LLP
ATTN: Kinal M. Patel, Esq.
1000 Louisiana St., Suite 2000
Houston, TX 77002

i. **Counterparts.** This Agreement may be executed in one or more counterparts, all of which together shall constitute only one agreement.

j. **Attorneys' Fees.** In the event that any legal action is instituted by any of the Parties hereto to enforce or construe any of the provisions or conditions of this Agreement, the Party prevailing in any such action shall be entitled to recover from the other Party all court costs and reasonable attorney fees, including the costs and fees incurred in enforcing any judgment.

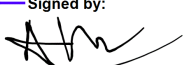
k. **Survival.** All provisions of this Agreement relating to intellectual property, limitation of liability, and indemnification, and any other provisions which must survive in order to give effect to their meaning, will survive the termination of this Agreement. The obligations of the receiving Party specified in Section 12 (Confidentiality) shall continue with respect to a particular item of Confidential Information until the fifth (5th) anniversary of the disclosure of such confidential information to the receiving Party hereunder; provided, however, that such obligations shall continue with respect to any confidential information that constitutes a trade secret until and unless one of the exceptions set forth in Section 12 (Confidentiality) applies to such item of confidential information.

[signature page follows]

IN WITNESS WHEREOF, the Parties hereto have caused this Agreement to be executed as of the Effective Date by their respective duly authorized officers.

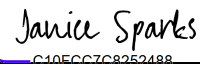
CLINIC:

[ISF]:

Signed by: 
By: _____
Name: Aslam Khan
Title: Chief Executive Officer
Date: 3/16/2026

CATCH:

CATCH Greater Houston


By: _____
Name: Janice Sparks
Title: Executive Director
Date: 3/11/2026

Program Appendix

Effective Date: [INSERT DATE]

PROGRAM: COMPREHENSIVE SPECIALTY NON-MEDICAL SUPPORT SERVICES VIA COMMUNITY HEALTH WORKERS

1. Program Overview

CATCH centralizes administrative, operational, and billing coordination functions to promote access to and integration of medical and non-medical care services (“NMS”) for individuals contacting the Harris County Area Agency on Aging (“AAA”) or another partner organization (“Community Partner”) who are seeking, or who may benefit from, community-based support services (each, a “Participant”).

NMS needs may include food insecurity, housing instability, transportation challenges, challenges with chronic illness, and other medical and social factors that impact health outcomes.

The primary goals of the Program are to streamline referral processes between and among community providers and resources, improve timely access to community-based and medical supports, and enhance care coordination using reimbursable Community Health Integration (“CHI”) and Principal Illness Navigation (“PIN”) services. These services are designed to address health-related social needs and assist patients with serious, high-risk conditions navigate the health care delivery system to improve outcomes.

The Program is constructed with the following workflow overview:

- a. If a Community Partner identifies a Participant seeking or potentially benefiting from community-based support services and enhanced care coordination, the Community Partner will conduct a preliminary, non-clinical screening for potential unmet NMS needs and verify insurance coverage at a high level before submitting a referral to CATCH. Referred Participants who complete intake and consent with CATCH are hereinafter referred to as “Program Members.”
- b. Upon receipt of a referral, CATCH will conduct intake and eligibility screening, including verification of Medicare coverage, completion of an Accountable Health Communities (“AHC”)-like screening tool, and physical and mental health screenings. If Program Member meets Program eligibility criteria and consents to services, CATCH will coordinate an initial medical visit, which may be conducted via telehealth, with a physician or non-physician practitioner (“Local Practitioner”) from a local health clinic partner (“Local Health Clinic Partner”) and introduce the Program Member to Community Health Workers (“CHWs”) supplied by a local health clinic or organization (“LHCO”).
- c. The CHW will accompany the Local Practitioner during a medical visit with the Program Member, which will include an AHC screening conducted by the Local Practitioner to identify linkages between the impacts of unmet NMS to negative health

outcomes. The Local Practitioner will create a care plan as appropriate for the Program Member.

- d. If the care plan incorporates CHI or PIN services, the LHCO CHW will implement the care plan and perform CHI and/or PIN services under the supervision of the Local Practitioner and in compliance with applicable third-party payor billing and documentation requirements.

The Program will be anchored to a single Local Health Clinic Partner – Ibn Sina Foundation. **For purposes of the Program, Ibn Sina Foundation will serve solely in its capacity as a Local Health Clinic Partner.**

CATCH may designate one or more replacement or new local health clinic partners for the Program, from time to time. No Local Health Clinic Partner is guaranteed any volume of Participants under the Program.

Further, with respect to Ibn Sina Foundation as a Local Health Clinic Partner, the Program is currently limited to Original Medicare and Medicare Advantage (including dual eligible individuals) and no other Payor Agreements.

2. Roles and Responsibilities

CATCH will serve as the coordinating specialty entity responsible for Program intake, eligibility determination, consent management, overall workflow coordination, billing support and monitoring, information management, outcomes analysis, and reporting. CATCH will maintain strong relationships with partner organizations, promote accurate and timely billing for CHI and PIN services, and support reporting and documentation compliance across all partners.

LHCO will provide CHWs who will deliver CHI and PIN services under the supervision of a Local Practitioner in accordance with CMS and other applicable Payor Agreement requirements. LHCO's CHWs will document all service encounters and outcomes in the Local Health Clinic Partner's electronic medical record system ("EHR").

Local Health Clinic Partners will function as the primary clinical anchor for medical services and will supervise and support CHI and PIN services, including approval of Intervention Plans and ongoing clinical oversight (together, "Clinical Services").

Program Members who request services, are screened for NMS, have legitimate healthcare needs, and who provide consent will be referred to the specialty service. Partner Organization staff will screen and verify insurance coverage, and a Partner Organization staff member will connect with the LHCO CHW. The CHW will confirm needs and input information into the EHR. The CHW will collaborate with the project care team to develop and monitor treatment plans.

3. Workflow Details

When a Program Member is identified by a Partner Organization with NMS needs, the Partner Organization will contact CATCH to facilitate a referral to a LHCO CHW. Partner Organization screening activities are preliminary and non-clinical and do not constitute eligibility

determinations for CHI or PIN services.

CATCH's intake of a referral from the Partner Organization will be recorded in CATCH's records system; and includes screening for and verifying Medicare plan coverage, completing an Accountable Health Communities (AHC)-like tool to identify NMS, and conducting physical and mental health screenings. If the Participant screens positive for unmet NMS needs and related health conditions, CATCH will offer enrollment in the Program, obtain written consent, and initiate referral to a Local Health Clinic Partner.

Simultaneous with a Local Practitioner, CATCH will coordinate telehealth appointments with a LHCO CHW for the Program Member. During the initiating visit with the CHW and Local Practitioner, the CHW, with support from CATCH, will complete the Accountable Health Communities tool, document NMS risks in the EHR, update the Program Member's problem list, and prepare treatment suggestions to reflect the impact of these social factors on the Program Member's health. Following the initial assessment, the CHW will coordinate with the Local Practitioner to develop a personalized intervention plan using CHI and PIN services. The Local Practitioner will create a care plan. The CHW will implement the plan under the supervision of the Local Practitioner and regularly update the Local Practitioner on patient progress.

Each service encounter will be documented by the CHW in the applicable Local Health Clinic Partner's EHR, including details of interventions, outcomes, and time spent per encounter. CATCH and Local Health Clinic Partner will coordinate on migration of records for such services to CATCH's records system pursuant to written consents.

4. Billing and Reconciliation

Consistent with Section 2(b) of the Agreement, all claims for CHI Services and PIN Services provided under the Program will be promptly billed by Clinic under Clinic's applicable Payor Agreements. All Actual Collections for such CHI Services and PIN Services shall belong exclusively to Clinic. CATCH shall not be deemed to have any ownership interest in such collections. As used in this Program Appendix, "Actual Collections" means amounts actually received by Clinic from applicable third-party payors (including patient cost-sharing amounts) for CHI Services and PIN Services performed under the Program and billed under Clinic's applicable Payor Agreements, net of refunds, chargebacks, recoupments, and retroactive payor adjustments.

5. Data and IT Integration

Program partners may use CATCH's EHR and data system that is accessible by all parties, according to CATCH's use or access standards.

6. Quality Assurance and Reporting

CATCH will establish regular meetings with all Program partners to review performance metrics, ensure quality assurance, and evaluate program improvement opportunities. Reports will include summaries of service delivery, billing outcomes, and barriers to access.

7. Deliverables

This Program will result in a finalized and operational referral workflow, accurate documentation and delivery of CHI and PIN services, monthly billing and revenue cycle reports, staff training materials, and an updated implementation timeline. All partner agreements will be executed to support successful implementation.

LHCO is responsible for ensuring all CHWs and other staff working for LHCO receive all required training as applicable to the Program(s) for which they are providing services.

Local Health Clinic Partner is responsible for ensuring all Local Practitioners and other staff working for Local Health Clinic Partner receive all required training as applicable to the Program(s) for which they are providing services; and for supervision and monitoring of CHW delivering CHI and PIN services to Program Members under personalized intervention plans.

CATCH may establish and maintain a continuous quality improvement (CQI) program. This will entail conducting regular quality assurance reviews and fidelity monitoring of services rendered by LHCO and Local Health Clinic Partner. LHCO and Local Health Clinic Partner will participate in good faith in the CQI program; and CATCH will provide information to LHCO and Local Health Clinic Partner on the outcome of the CQI program and discuss recommended areas of improvement.

8. Lead Managers

Within ten (10) days following the Effective Date of this Statement of Work, the Parties shall each designate a representative to serve as the manager of such Party's responsibilities with respect to LHCO's and Local Health Clinic Partner's participation in the Program (each, a "Lead Manager"). The Lead Managers shall hold an organizational meeting within ten (10) days after being selected as the Lead Manager to establish a project plan and timeline for operationalizing and monitoring LHCO's and Local Health Clinic Partner's participation in the Program. The Lead Project Managers shall meet regularly thereafter.

9. Practice of Healthcare Professions

As between CATCH and LHCO, LHCO is solely responsible for the practice of health care professionals and personnel (including CHWs) of LHCO HCPs, and the delivery of CIN and PIN Services, hereunder.

As between CATCH and Local Health Clinic Partner, Local Health Clinic Partner is solely responsible for the practice of health care professionals and personnel (including Local Practitioners), and the supervision of CIN and PIN Services, hereunder.

10. Compensation

As full and complete consideration for the services provided by CATCH under the Agreement and this Program Appendix, including without limitation the provision of community health worker services (directly or through its partners), care coordination, billing and collection, data management, reporting, and administrative support, Clinic shall pay to CATCH an amount equal

to eighty percent (90%) of Actual Collections received by Clinic for CHI Services and PIN Services provided under the Program. Such payment shall be made within thirty (30) days following Clinic's receipt of the applicable Actual Collections.

For the avoidance of doubt, CHI Services and PIN Services compensated under this Section 10 are limited solely to services furnished pursuant to the Program and billed under Clinic's applicable Payor Agreements, regardless of whether particular service components are performed by community health workers, Clinic personnel, or other Program participants, and do not include any other medical, clinical, or professional services furnished by Clinic.

As of the Effective Date of this Program Appendix, CHI Services include services billed under the following HCPCS codes, as approved for reimbursement by the applicable Payor: G0019, G0022, G0023, G0024, G0136, G0140, and G0146; and PIN Services include services billed under the following HCPCS codes, as approved for reimbursement by the applicable Payor: G0019, G0022, G0023, G0024, G0140, and G0146. The Parties may amend the foregoing list of HCPCS codes by mutual written agreement in response to changes in applicable Payor requirements, without amending the Agreement.

The Parties acknowledge that reimbursement rates, regulatory requirements, and Program scope may evolve over time. Accordingly, beginning on the date that is twelve (12) months following the Effective Date of this Program Appendix, and no more frequently than annually thereafter, the Parties may, by mutual written agreement, revise the compensation set forth in this Section 10. Any such revised compensation shall be commercially reasonable, consistent with fair market value, and shall not be determined in a manner that takes into account the volume or value of referrals or other business generated between the Parties, except as permitted by applicable law. Any such revision shall be reflected solely in an amendment to this Program Appendix and shall not require amendment of the Agreement.

EXHIBIT B

Business Associate Agreement

This Business Associate Agreement (“BAA”) is entered into effective this 12th day of March, 2026 (“Effective Date”) by and between **Ibn Sina Foundation** on behalf of all members of its affiliated covered entity (“Covered Entity”) and CATCH Greater Houston (“Business Associate”) (each a “Party” and collectively, the “Parties”).

RECITALS

WHEREAS, Business Associate performs certain services for or on behalf of Covered Entity, and in performing said services, Business Associate creates, receives, maintains, or transmits Protected Health Information (“PHI”);

WHEREAS, the Parties intend to protect the privacy and provide for the security of the PHI Disclosed (as defined below) by Covered Entity to Business Associate, or created, received, maintained, or transmitted by Business Associate, when providing services. Such PHI will be protected in compliance with the Health Insurance Portability and Accountability Act (“HIPAA”), the Health Information Technology for Economic and Clinical Health Act (Public Law 111-005) (the “HITECH Act”) and its implementing regulations and guidance issued by the Secretary of the U.S. Department of Health and Human Services (“Secretary”) (collectively, the “HIPAA Regulations”); and

WHEREAS, Covered Entity is required under the HIPAA Regulations to enter into a Business Associate Agreement that meets certain requirements with respect to the Use (as defined below) and Disclosure of PHI, which are met by this BAA. Accordingly, to the extent Business Associate is functioning as a “business associate” as defined in the HIPAA Regulations, Business Associate agrees to comply with this BAA.

In consideration of the Recitals and for other good and valuable consideration, the receipt and adequacy of which is hereby acknowledged, the Parties agree as follows:

DEFINITIONS

The following terms shall have the respective meanings set forth below. Capitalized terms used in this BAA and not otherwise defined shall have the meanings ascribed to them in the HIPAA Regulations.

“**Breach**” shall have the meaning given to such term under 45 C.F.R. § 164.402.

“**Designated Record Set**” shall have the meaning given to such term under 45 C.F.R. § 164.501.

“**Disclose**” and “**Disclosure**” mean, with respect to PHI, the release, transfer, provision of access to, or divulging in any other manner of PHI outside of Business Associate or to other than members of its Workforce, as set forth in 45 C.F.R. § 160.103.

“**Electronic PHI**” or “**e-PHI**” means PHI that is transmitted or maintained in electronic media, as set forth in 45 C.F.R. § 160.103.

“**Protected Health Information**” and “**PHI**” mean any information, whether oral or recorded in any form or medium, provided by Covered Entity to Business Associate, that: (a) relates to the past, present or future physical or mental health or condition of an individual; the provision of health care to an individual, or the past, present or future payment for the provision of health care to an individual; (b) identifies the individual (or for which there is a reasonable basis for believing that the information can be used to identify the individual); and (c) shall have the meaning given to such term under 45 C.F.R. § 160.103. Protected Health Information includes e-PHI.

“**Required by Law**” shall have the meaning given to such term under 45 C.F.R. § 164.103.

“**Security Incident**” shall have the meaning given to such term under 45 C.F.R. § 164.304.

“**Services**” shall mean the services or functions performed by Business Associate for or on behalf of Covered Entity pursuant to any service agreement(s) between Covered Entity and Business Associate which may be in effect now or from time to time

“**Underlying Agreement**”), or, if no such agreement is in effect, the services or functions performed by Business Associate that constitute a “business associate” relationship, as set forth in 45 C.F.R. § 160.103.

“**Unsecured PHI**” shall have the meaning given to such term under 42 U.S.C. § 17932(h), 45 C.F.R. § 164.402, and guidance issued pursuant to the HITECH Act including, but not limited to the guidance issued on April 17, 2009 and published in 74 Federal Register 19006 (April 27, 2009) by the Secretary.

“**Use**” or “**Uses**” mean, with respect to PHI, the sharing, employment, application, utilization, examination or analysis of such PHI within Business Associate’s internal operations, as set forth in 45 C.F.R. § 160.103.

“**Workforce**” shall have the meaning given to such term under 45 C.F.R. § 160.103.

OBLIGATIONS OF BUSINESS ASSOCIATE

2.1. Permitted Uses and Disclosures of PHI. Business Associate shall not Use or Disclose PHI created, received, maintained, or transmitted for or on behalf of Covered Entity except to perform the Services required by the Underlying Agreement, or as permitted by this BAA or Required by Law. Business Associate shall not Use or Disclose PHI in any manner that would constitute a violation of the HIPAA Regulations if so Used or Disclosed by Covered Entity. Without limiting the generality of the foregoing, Business Associate is permitted to (i) Use and Disclose PHI for the proper management and administration of Business Associate or to carry out the legal responsibilities of Business Associate, provided that with respect to any such Disclosure either: (a) the Disclosure is Required by Law; or (b) Business Associate obtains agreement from the person to whom the PHI is to be Disclosed that such person will hold the PHI in confidence and will not Use or further Disclose such PHI except as Required by Law and for the purpose(s) for which it was Disclosed by Business Associate to such person, and that such person will notify Business Associate of any instances

of which it is aware in which the confidentiality of the PHI has been breached; (ii) Use PHI for Data Aggregation purposes in connection with the Health Care Operations of Covered Entity; and (iii) Use PHI to create de-identified information in accordance with the requirements outlined in the HIPAA Regulations. Data that has been de-identified will no longer be subject to the terms of this BAA.

2.2. Adequate Safeguards of PHI. Business Associate agrees to use appropriate safeguards and comply, where applicable, with Subpart C of 45 C.F.R. Part 164 with respect to e-PHI, to prevent use or disclosure of the information other than as provided for by this BAA.

2.3. Mitigation. Business Associate agrees to mitigate, to the extent practicable, any harmful effect that is known to Business Associate of a Use or Disclosure of PHI by Business Associate in violation of the requirements of this BAA.

2.4. Reporting Breaches, Security Incidents, and Non-Permitted Uses or Disclosures. Business Associate shall notify Covered Entity of any Use or Disclosure by Business Associate or its Subcontractors that is not permitted by this BAA and each Security Incident, including Breaches of Unsecured PHI, within five (5) business days of discovery. Notwithstanding the foregoing, Business Associate and Covered Entity acknowledge the ongoing existence and occurrence of attempted but ineffective Security Incidents that are trivial in nature, such as pings and other broadcast service attacks, and Covered Entity acknowledges and agrees that no additional notification to Covered Entity of such ineffective Security Incidents is required, as long as no such incident results in unauthorized access, Use or Disclosure of PHI. If Business Associate determines that a Breach of Unsecured PHI has occurred, Business Associate shall provide a written report to Covered Entity without unreasonable delay and no later than thirty (30) calendar days after discovery of the Breach. To the extent that information is available to Business Associate, Business Associate’s written report to Covered Entity shall be in accordance with 45 C.F.R. § 164.410(c).

2.5. Delegated Responsibilities. To the extent that Business Associate carries out one or more of Covered Entity's obligations under Subpart E of 45 C.F.R. Part 164, Business Associate must comply with the requirements of Subpart E that apply to Covered Entities in the performance of such obligations.

2.6. Availability of Internal Practices, Books, and Records to Government. Business Associate agrees to make its internal practices, books and records relating to the Use and Disclosure of Covered Entity's PHI available to the Secretary for purposes of determining Covered Entity's compliance with the HIPAA Regulations.

2.7. Access to and Amendment of Protected Health Information. To the extent that Business Associate maintains a Designated Record Set on behalf of Covered Entity, Business Associate shall (i) make the PHI it maintains (or which is maintained by its Subcontractors) in such Designated Record Set available to Covered Entity for inspection and copying to enable Covered Entity to fulfill its obligations under 45 C.F.R. § 164.524 within fifteen (15) business days of a written request by Covered Entity; and (ii) amend the PHI it maintains (or which is maintained by its Subcontractors) in such Designated Record Sets to enable the Covered Entity to fulfill its obligations under 45 C.F.R. § 164.526 within fifteen (15) business days of a written request by Covered Entity.

2.8. Accounting. Business Associate agrees to document such disclosures of PHI and information related to such disclosures as would be required for Covered Entity to respond to a request by an individual for an accounting of disclosures of PHI in accordance with 45 C.F.R. § 164.528. Business Associate shall provide to Covered Entity or, at the request of Covered Entity directly to an individual, in the time and manner designated by Covered Entity, but in no event longer than fifteen (15) days after Business Associate's receipt of a written request from Covered Entity, information collected in accordance this Section 2.8 of this BAA, to permit Covered Entity to respond to a request by an individual for an accounting of disclosures of PHI in accordance with 45 C.F.R. § 164.528.

2.9. Use of Subcontractors. Business Associate shall require each of its Subcontractors that creates, receives, maintains, or transmits PHI on behalf of Business Associate, to execute a written agreement that includes substantially the same restrictions and conditions that apply to Business Associate under this BAA with respect to PHI.

2.10. Minimum Necessary. Business Associate (and its Subcontractors) shall, to the extent practicable, limit its request, Use, or Disclosure of PHI to the minimum amount of PHI necessary to accomplish the purpose of the request, Use or Disclosure, in accordance with 42 U.S.C. § 17935(b) and 45 C.F.R. § 164.502(b)(1) or any other guidance issued thereunder.

TERM AND TERMINATION

3.1. Term. The term of this BAA shall be effective as of the Effective Date and shall remain in effect until all of the PHI provided by Covered Entity to Business Associate, or created or received by Business Associate on behalf of Covered Entity, is destroyed or returned to Covered Entity, or, if it is infeasible to return or destroy PHI, protections are extended to such information, in accordance with Section 3.3.

3.2. Termination for Cause. In addition to and notwithstanding the termination provisions set forth in any Underlying Agreement, upon Covered Entity's or Business Associate's knowledge of a material breach or violation of this BAA by the other Party, the non-breaching Party shall notify the breaching Party of the breach in writing, and provide an opportunity for the breaching Party to cure the breach or end the violation within thirty (30) days of such notification; provided that if the breaching Party fails to cure the breach or end the violation within such time period to the satisfaction of the non-breaching Party, the non-breaching Party may immediately terminate this BAA upon written notice to the breaching Party.

3.3. Disposition of PHI Upon Termination. Upon termination or expiration of this BAA, Business Associate shall either return or destroy all PHI

received from, or created or received by Business Associate on behalf of Covered Entity, that Business Associate still maintains in any form and retain no copies of such PHI. If return or destruction is not feasible, Business Associate shall continue to extend the protections of this BAA to the PHI for as long as Business Associate retains the PHI and limit further Uses and Disclosures of such PHI to those purposes that make the return or destruction of the PHI infeasible.

MISCELLANEOUS

4.1. Covered Entity's Obligations. Covered Entity shall notify Business Associate in writing of any of the following, to the extent that such limitation, change, revocation, or restriction may affect Business Associate's Use or Disclosure of PHI: (i) any limitation(s) in Covered Entity's notice of privacy practices; (ii) any changes in, or revocation of, permission by an individual to Use or Disclose PHI; or (iii) any restriction to the Use or Disclosure of PHI that Covered Entity has agreed to in accordance with 45 C.F.R. § 164.522. Covered Entity will obtain any consent or authorization that may be required by the HIPAA Regulations, or applicable state law, prior to furnishing Business Associate with PHI.

4.2. Amendment to Comply with Law. To the extent applicable, amendments or modification to the HIPAA Regulations may require amendments to certain provisions of this BAA. Amendments shall only be effective if executed in writing and signed by a duly authorized representative of each Party.

4.3. Relationship to Underlying Agreement Provisions. In the event that a provision of this BAA is contrary to a provision of an Underlying Agreement, the provision of this BAA shall control. Otherwise, this BAA shall be construed under, and in accordance with, the terms of such Underlying Agreement, and shall be considered an amendment of and supplement to such Underlying Agreement.

4.4. Notices. Any notices or communications hereunder shall be in writing by certified mail, return receipt requested, or delivered by a nationally

recognized courier service with delivery confirmation, such as FedEx, or by facsimile (with evidence of receipt) at the addresses that follow the signature blocks at the end of this BAA.

4.5. Relationship of Parties. Notwithstanding anything to the contrary in any Underlying Agreement, Business Associate is an independent contractor and not an agent of Covered Entity under this BAA. Business Associate has the sole right and obligation to supervise, manage, contract, direct, procure, perform or cause to be performed all Business Associate obligations under this BAA.

4.6. Interpretation. This BAA shall be interpreted as broadly as necessary to implement and comply with the HIPAA Regulations. The Parties agree that any ambiguity in this BAA shall be resolved in favor of a meaning that complies and is consistent with such laws and regulations.

4.7. Regulatory References. A reference in this BAA to a section in the HIPAA Regulations means the section as in effect or as amended, and for which compliance is required.

4.8. No Third Party Beneficiaries. Nothing express or implied in this BAA is intended to confer, nor shall anything herein confer, upon any person other than the Parties and the respective successors or assigns of the Parties, any rights, remedies, obligations, or liabilities whatsoever.

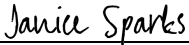
4.9. Counterparts; Electronic Signatures. This BAA may be executed in one or more counterparts, all of which together shall constitute only one agreement. If any signature is delivered by facsimile or email or is signed in any electronic format, such signature shall create a valid and binding obligation with the same force and effect as if such signature were handwritten.

The Parties hereto have executed this BAA as of the Effective Date.

For BUSINESS ASSOCIATE:

CATCH Greater Houston

Signed by:



By: Janice Sparks

Title: Executive Director

Dated: 3/12/26

190 Heights Boulevard

Houston, TX 77007

fax:832-393-5214

email: janice.sparks@catchgh.org

For COVERED ENTITY:

Ibn Sina Foundation



By:

Title:

Dated: 3/16/2026

Aslam Ghalib
[insert address]

CEO

fax: _____

email: ceo@ibnsinafoundation.org

